

Policy on Information on Expected Costs

«Hospital Name»

Document control

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Date created	_____	Date of implementation	_____
Prepared by	Patient Accounts In-Charge Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

A blank marked _____ must be completed before issue.

Statement of intent

Patients and families at «Hospital Name» have a right to information on expected costs, so they can make informed decisions about their care without surprise charges.

1. Purpose

This policy sets out how «Hospital Name» makes its pricing policy and tariff available to patients and families, explains expected treatment costs, and informs them of financial implications when the care plan changes.

This policy does not cover informed consent, patient education about healthcare needs, or complaint redressal — those are covered in other hospital policies.

2. Scope

This policy applies to all patients and families across out-patient, emergency, ICU and in-patient settings at «Hospital Name».

3. Policy standards

«Hospital Name» makes patients and families aware of the pricing policy in each setting — out-patient, emergency, ICU and in-patient — and makes the relevant tariff list available to patients.

«Hospital Name» explains expected costs to patients and families and informs them about financial implications when there is a change in the care plan.

4. Non-negotiable rules

- Do not leave the pricing policy undisplayed at the registration or admission desk for any setting — out-patient, emergency, ICU or in-patient.
- Do not charge outside the current tariff list, and do not leave an additional charge unenumerated or uncommunicated to the patient.
- Give every patient an estimate of expected treatment costs, based on the treatment plan.
- Do not leave a patient or family uninformed of the financial implications when the care plan changes.
- Staff who see a rule under this policy broken report it the same shift to the Patient Accounts In-Charge or the Medical Superintendent.

5. What we do

5.1 Pricing policy display

Key components of pricing — at minimum consultation charges, bed charges, nursing charges and security deposit — are available to patients near the registration and/or admission desk at «Hospital Name», based on the billing policy that defines charges for various healthcare activities. Pricing information is available for out-patient, emergency, ICU and in-patient settings.

5.2 Tariff list availability

«Hospital Name» maintains an updated tariff list, available for review to patients when required. Charges follow the tariff list; any additional charge is enumerated in the tariff and communicated to patients. Tariff rates are uniform within a given setting and transparent.

5.3 Cost estimates

Patients at «Hospital Name» are given an estimate of expected treatment expenses, based on the treatment plan, preferably in written form. The estimate may be prepared by OPD, registration or admission staff in consultation with the treating doctor. Any limitations of the estimate — for example for emergency admissions — may also be discussed with the patient.

5.4 Financial implications of care plan changes

When there is a change in the care plan at «Hospital Name» — for example a shift between settings such as to or from the ICU, a shift from medical to surgical management, or the need for further expensive investigations — the patient and/or family are informed of the financial implications.

6. Stop-work authority

Not applicable. This standard has no stop-work authority — gaps in cost information are addressed through the hospital's quality monitoring and CAPA process (Section 8), not through an immediate stop-work trigger.

7. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable for ensuring patients and families receive cost information as required by this policy; receives escalations under Section 4.
Patient Accounts In-Charge	Owns implementation of this policy; maintains the billing policy, pricing display and tariff list; keeps these current.
OPD / Registration / Admission Staff	Prepare cost estimates in consultation with the treating doctor; direct patients to pricing and tariff information.
Treating Doctor	Confirms the treatment plan behind each cost estimate; informs the patient and family of care-plan changes affecting cost.
Quality Coordinator	Audits this policy; holds training records and staff acknowledgements.

8. Quality monitoring

Monitoring area	What is monitored
Pricing policy display	Displayed pricing-policy document (with date); billing policy it is based on; audit or observation record confirming display is in place across settings.
Tariff list availability	Dated tariff list (current version); charge-versus-tariff reconciliation record for a sampled bill; confirmation no undisclosed charge was applied.
Cost estimates	Cost-estimate records given to patients (written where possible), filed in the patient account or medical record; record of limitations discussed.
Care plan change communication	Revised-estimate or cost-change notification records; confirmation no surprise charge appeared at discharge for a known plan change.

9. Training and staff acknowledgement

All staff covered by this policy are trained on how to direct patients to pricing and cost information and on the estimate and care-plan-change notification requirements, at induction and periodically.

I have read the Policy on Information on Expected Costs of «Hospital Name» and understand my responsibilities under it.

Staff name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

10. Distribution

This policy shall be available to the Patient Accounts department, OPD and Admission desks, treating doctors, and all staff through the hospital intranet and department policy folders.

11. Abbreviations

Abbreviation	Meaning
CAPA	Corrective and Preventive Action
ICU	Intensive Care Unit
NABH	National Accreditation Board for Hospitals and Healthcare Providers
OPD	Out-Patient Department

12. Traceability table

This table is an index. It is not how the policy is organised. An asterisk in the Level column means the objective element is starred and requires documented evidence.

Objective Element	Level	Traceability to this policy
PRE.6.a	CORE	Sections 3 and 5.1 address display of pricing policy components across out-patient, emergency, ICU and in-patient settings.
PRE.6.b	Commitment	Sections 3 and 5.2 address availability of an updated tariff list and charging as per the tariff.
PRE.6.c	Commitment	Sections 3 and 5.3 address explanation of expected treatment costs, preferably in written form, based on

		the treatment plan.
PRE.6.d	Commitment	Sections 3 and 5.4 address informing the patient and/or family of the financial implications of a care plan change.

13. Required Records/Evidence Checklist

Pricing policy display — PRE.6.a (CORE)

- Displayed pricing-policy document (with date) and the billing policy it is based on.
- Audit or observation record confirming the display is in place at registration and/or admission desk across all applicable settings.

Tariff list — PRE.6.b (Commitment)

- Dated tariff list (current version).
- Charge-versus-tariff reconciliation record for a sampled bill.
- Confirmation that no undisclosed additional charge was applied.

Cost estimates — PRE.6.c (Commitment)

- Cost-estimate records given to patients (written where possible), filed in the patient account or medical record.
- Records of any limitations discussed (for example for emergency admissions).

Care plan change financial communication — PRE.6.d (Commitment)

- Records of revised-estimate or cost-change notification when the care plan shifted, noting the change and the date the patient or family was informed.
- Confirmation no surprise discharge charge occurred for a known plan change.

14. References

1. National Accreditation Board for Hospitals and Healthcare Providers. NABH Accreditation Standards for Hospitals, 6th Edition. PRE.6.

Disclaimer

This policy reorganises the supplied PRE.6 objective-element wording and Guidebook interpretation into plain-language guidance for «Hospital Name». It does not replace the NABH Accreditation Standards for Hospitals, 6th Edition, or the Guidebook, which remain the authoritative source in case of any conflict.

This policy is intended for accreditation and internal governance purposes and is not a statement of, or substitute for, compliance with any specific statute; «Hospital Name»

remains separately responsible for meeting all applicable statutory and regulatory requirements.

This policy shall be reviewed at least once every year, or earlier if NABH standards change.

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